

CLINICAL ASSESSMENT REPORT

Greenfield Medical Practice — Cardiology Outpatient Clinic

PATIENT NAME: Thomas Wright
DATE OF BIRTH: 19/07/1968 (Age: 57 years)
NHS NUMBER: NHS-456 789 1234
NEXT REVIEW: 28 July 2026

PATIENT ID: PT-2026-05589
GENDER: Male
CLINIC DATE: 28 April 2026
CONSULTING PHYSICIAN: Dr. Claire Montague

REASON FOR VISIT / CHIEF COMPLAINT

Follow-up appointment for hypertension management and medication review. Patient reports persistent morning headaches (3/10 severity), occasional exertional chest tightness (NYHA Class II), and fatigue over the past 4 weeks. Denies syncope, palpitations, or peripheral oedema.

MEDICAL HISTORY

Hypertension: Diagnosed 2018, on treatment

Hypercholesterolaemia: Diagnosed 2020

Type 2 Diabetes Mellitus: Diagnosed 2022, diet-controlled + oral agents

Obesity: BMI 31.4 kg/m²

Ex-Smoker: 15 pack-year history, quit 2019

CURRENT MEDICATIONS

MEDICATION	DOSE	FREQUENCY / NOTES
Amlodipine	10 mg	Once daily (morning)
Ramipril	5 mg	Once daily (morning)
Atorvastatin	40 mg	Once daily (night)
Metformin	1000 mg	Twice daily with meals
Aspirin	75 mg	Once daily (morning)
Bisoprolol	2.5 mg	Once daily (morning) — newly added today

VITAL SIGNS

TEST	RESULT	UNIT	REFERENCE RANGE	FLAG
Blood Pressure (sitting)	158 / 96	mmHg	< 130 / 80	HIGH
Blood Pressure (standing, 3 min)	152 / 92	mmHg	< 130 / 80	HIGH

Heart Rate

bpm

60 – 100

Oxygen Saturation (SpO ₂)	97	%	≥ 95%	
Respiratory Rate	16	breaths/min	12 – 20	
Temperature	36.7	°C	36.1 – 37.2	
Weight	94	kg		
BMI	31.4	kg/m ²	18.5 – 24.9	HIGH
Fasting Blood Glucose (today)	138	mg/dL	70 – 99	HIGH

PHYSICAL EXAMINATION

General: Alert, orientated, mildly overweight. No distress at rest.

Cardiovascular: Regular rate and rhythm. S1 S2 heard. Soft ejection systolic murmur at aortic area (grade II/VI) — unchanged from prior. No added sounds. No carotid bruits. JVP not elevated. No peripheral oedema.

Respiratory: Clear to auscultation bilaterally. No wheeze or crepitations.

Abdomen: Soft and non-tender. No hepatosplenomegaly. No abdominal bruit.

Neurological: Grossly intact. No focal neurological deficit.

INVESTIGATIONS ORDERED

1. Fasting lipid profile + HbA1c
2. U&E, eGFR, LFTs, TFTs
3. 12-lead ECG (performed today — sinus rhythm, LVH pattern noted)
4. Ambulatory 24-hour blood pressure monitoring
5. Echocardiogram (to assess LV hypertrophy given ECG changes)
6. Urine ACR (albumin-creatinine ratio) for diabetic nephropathy screening

ASSESSMENT AND PLAN

Primary Diagnosis: Poorly controlled essential hypertension with features of hypertensive end-organ damage (ECG LVH).

Secondary Diagnoses: Type 2 diabetes mellitus with suboptimal glycaemic control (fasting glucose 138 mg/dL — requires HbA1c review). Hypercholesterolaemia on atorvastatin.

Management Plan:

1. Bisoprolol 2.5 mg OD added to regimen for rate control and BP management.
2. Reinforce lifestyle modification: DASH diet, sodium restriction (< 2 g/day), weight loss target 5 kg.
3. Review HbA1c result — if > 8%, consider escalation of diabetic therapy (add SGLT2i / DPP-4 inhibitor).
4. Repeat BP check in 4 weeks; target BP < 130/80 mmHg.
5. Echocardiogram referral to assess LV function and rule out hypertensive cardiomyopathy.
6. Patient counselled regarding cardiovascular risk factors and signs of hypertensive emergency.

